

Aarogya360

Product Requirements Document (PRD)

Healthcare SaaS Platform for Clinics, Doctors, and Patients

Aarogya360 Product Team

Version 1.0
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1. Document Information

1.1 Product Name

Aarogya360 – a healthcare SaaS platform unifying clinic operations, doctor workflows, and patient health record ownership.

1.2 Version History

Version	Date	Author	Summary of Changes
0.1	TBD	Product Team	Initial blueprint drafted
1.0	June 19, 2026	Product Team	First complete PRD covering all modules, AI requirements, and roadmap

1.3 Stakeholders

- Founder / Product Owner
- Engineering Lead (Backend)
- Engineering Lead (Frontend / Mobile)
- AI / ML Lead
- Clinical Advisor (Practicing Doctor)
- UI/UX Designer
- QA Lead
- DevOps / Infrastructure Owner
- Compliance / Legal Advisor

1.4 Approval Matrix

Role	Name	Approval Status	Date
Product Owner		Pending	
Engineering Lead		Pending	
Clinical Advisor		Pending	
Compliance Advisor		Pending	

2. Executive Summary

2.1 Vision

A patient's health history should never be lost. Aarogya360 envisions a connected healthcare ecosystem across India where every consultation, prescription, and lab report becomes a permanent, portable part of a patient's lifelong health record – accessible regardless of which clinic they visit.

2.2 Mission

To digitize the complete clinical workflow for small and medium clinics, while giving every patient lifetime ownership and access to their own health data through a secure, AI-enabled mobile health passport.

2.3 Product Positioning

Aarogya360 positions itself as an affordable, easy-to-adopt clinic management system for India's small and medium clinics – distinct from large, complex hospital information systems (HIS) that are too expensive or heavy for single-doctor and small multi-doctor practices. Its differentiator is the combination of:

- A lightweight, modular clinic dashboard
- A patient-owned mobile health passport
- Practical, problem-solving AI features (not AI for its own sake)
- A clear path toward national health ecosystem integration (ABHA)

2.4 Market Opportunity

India has a large base of small and medium clinics still operating on paper records, manual registers, and phone-based reminders. The push toward the Ayushman Bharat Digital Mission (ABDM) and ABHA-linked health records creates a regulatory and infrastructural tailwind for digitization. Clinics that digitize early gain better patient retention, operational efficiency, and readiness for future national health data integration mandates.

3. Problem Statement

3.1 Clinic Problems

- Paper-based patient records that are difficult to retrieve, organize, or back up
- Manual appointment management leading to overbooking, long wait times, and disorganized queues
- Frequently lost or illegible prescriptions
- No structured mechanism for follow-up or recall tracking
- Poor patient retention due to lack of continuity and communication
- Lack of operational and financial insight into clinic performance

3.2 Doctor Problems

- Time lost manually writing prescriptions and notes during consultations
- No quick access to a patient's complete history, chronic conditions, or current medications
- Risk of prescribing drugs with dangerous interactions without an automated check
- Difficulty tracking long-term patient outcomes across multiple visits

3.3 Patient Problems

- Health records scattered across multiple clinics and providers
- Difficulty tracking which medicines were prescribed and when
- Difficulty understanding lab reports and medical terminology
- No centralized, lifelong view of personal health history
- No easy way to revisit past consultations or prescriptions

4. Product Goals

4.1 Business Goals

- Acquire and retain small and medium clinics as paying subscribers
- Establish Aarogya360 as a trusted, production-grade clinic management platform
- Build a defensible data and AI moat through accumulated, consented patient health data
- Create a scalable subscription-based revenue model

4.2 User Goals

- Clinics: eliminate paper records and reduce administrative overhead
- Doctors: save time per consultation and reduce clinical errors
- Patients: gain lifetime, portable access to their own health history
- Receptionists and Lab Staff: streamline daily operational tasks

4.3 Technical Goals

- Build a modular, maintainable, and scalable backend architecture
- Ensure strict multi-tenant data isolation between clinics
- Deliver AI features that are accurate, explainable, and advisory in nature
- Maintain high availability, strong security, and regulatory readiness (ABHA/ABDM)

5. Scope

5.1 In Scope

- Clinic Dashboard (web application) for clinic owners, doctors, receptionists, and lab staff
- Patient Mobile Application functioning as a lifelong health passport
- Core clinical workflow: patient management, appointments, consultations, prescriptions, lab orders, billing
- AI-powered features: OCR, health summaries, lab analysis, drug interaction detection, patient AI assistant
- Multi-tenant architecture with consent-based cross-clinic record sharing
- ABHA (Ayushman Bharat Health Account) readiness and integration

5.2 Out of Scope

- Telemedicine / video consultations (future vision)
- Pharmacy inventory and fulfillment integration (future vision)
- Insurance claim processing and integration (future vision)
- Hospital-grade modules such as OT scheduling, in-patient bed management, or ICU monitoring
- Support for languages beyond initially targeted regional languages in Phase 1–5

6. User Personas

6.1 Clinic Owner

Goals: Reduce operational overhead, retain patients, grow revenue, gain visibility into clinic performance.

Pain Points: Paper records, no analytics, inconsistent staff processes, billing leakage.

6.2 Doctor

Goals: Spend more time with patients and less on paperwork, avoid prescribing errors, track patient history quickly.

Pain Points: Manual record-keeping, no access to past consultations across visits, no drug interaction safety net.

6.3 Receptionist

Goals: Manage patient registration and appointment scheduling efficiently.

Pain Points: Manual registers, double-booked slots, long queues, repetitive data entry.

6.4 Lab Technician

Goals: Process lab orders and upload results quickly and accurately.

Pain Points: Disconnected lab order tracking, manual result entry, no automated abnormality flagging.

6.5 Patient

Goals: Easily access personal health history, understand prescriptions and lab reports, manage family health records.

Pain Points: Scattered records, lost prescriptions, confusing medical terminology, no single source of truth.

6.6 System Administrator

Goals: Ensure platform uptime, security, tenant provisioning, and compliance.

Pain Points: Manual tenant onboarding, lack of centralized monitoring and audit visibility.

7. User Journey Maps

7.1 Patient Journey

1. Patient is registered at a clinic (in person or via QR profile linking)
2. Patient books or is assigned an appointment and receives a reminder
3. Patient is seen by a doctor; consultation, diagnosis, and prescription are recorded digitally
4. Patient receives a digital prescription and, if applicable, lab orders
5. Lab results are uploaded and patient is notified
6. Patient views the entire visit timeline in the mobile app's Health Passport
7. Patient can ask the AI Health Assistant questions about their own records at any time

7.2 Doctor Journey

1. Doctor logs into the Clinic Dashboard and views the day's OPD queue
2. Doctor opens a patient profile and reviews AI-generated health summary (history, chronic conditions, current medications)
3. Doctor records symptoms, examination findings, and diagnosis
4. Doctor creates a digital prescription; system flags any drug interaction warnings
5. Doctor orders lab tests if required and moves to the next patient

7.3 Receptionist Journey

1. Receptionist registers a new patient or searches for an existing one
2. Receptionist books an appointment and generates a token for the OPD queue
3. Receptionist manages walk-ins and queue order throughout the day
4. Receptionist handles billing and payment collection after consultation

7.4 Clinic Owner Journey

1. Clinic owner onboards the clinic and configures staff accounts and roles
2. Clinic owner monitors daily operations through the analytics dashboard

3. Clinic owner reviews revenue reports and patient retention metrics
4. Clinic owner manages subscription plan and billing for the platform itself

8. Functional Requirements

8.1 Authentication Module

8.1.1 Registration

Support registration of clinics and staff accounts with role assignment at creation time.

8.1.2 Login

Secure login via email/phone and password, with JWT-based session issuance.

8.1.3 Password Reset

Self-service password reset via OTP (SMS/email) with secure, time-limited reset tokens.

8.1.4 MFA

Optional multi-factor authentication (OTP-based) for clinic owners and doctors handling sensitive data.

8.1.5 Session Management

Token expiry, refresh token rotation, and the ability to revoke active sessions.

8.2 Multi-Tenancy Module

8.2.1 Tenant Isolation

Each clinic's data is logically isolated; no clinic can access another clinic's records under any circumstance.

8.2.2 Tenant Provisioning

Automated onboarding flow to provision a new clinic tenant, including default roles and configuration.

8.2.3 Subscription Plans

Support for tiered subscription plans (e.g., Basic, Pro, Enterprise) governing feature access and usage limits.

8.3 Clinic Module

8.3.1 Clinic Registration

Capture clinic details: name, address, specialty, operating hours, and branding.

8.3.2 Staff Management

Add, remove, and manage roles for doctors, receptionists, and lab staff.

8.3.3 Department Management

Organize doctors and services into departments/specialties for larger multi-doctor clinics.

8.4 Doctor Module

8.4.1 Doctor Profiles

Specialty, qualifications, consultation fee, and profile details.

8.4.2 Availability

Doctors configure available slots, leave, and working days.

8.4.3 Scheduling

System auto-allocates appointment slots based on doctor availability and existing bookings.

8.5 Patient Module

8.5.1 Registration

Capture demographic and contact details; generate a unique patient ID/QR profile.

8.5.2 Search

Fast search by name, phone number, or patient ID across the clinic's patient base.

8.5.3 Health Passport

A consolidated, chronological view of all consultations, prescriptions, and lab reports for a patient.

8.5.4 Family Linking

Link family members under a single patient account for shared visibility and management.

8.6 Appointment Module

8.6.1 Booking

Patients or staff can book appointments against doctor availability.

8.6.2 Rescheduling

Appointments can be rescheduled or cancelled with audit tracking.

8.6.3 Queue Management

Live OPD queue with token-based sequencing and real-time status.

8.6.4 Notifications

Automated reminders for upcoming appointments via SMS/push notifications.

8.7 Consultation Module

8.7.1 Symptoms

Structured capture of patient-reported symptoms.

8.7.2 Examination

Structured capture of clinical examination findings.

8.7.3 Diagnosis

Doctor records diagnosis, optionally coded against standard terminologies (e.g., ICD-10).

8.7.4 Treatment Plans

Doctor documents the treatment plan and follow-up instructions.

8.8 Prescription Module

8.8.1 Digital Prescriptions

Doctors create structured, digital prescriptions replacing handwritten ones.

8.8.2 Templates

Reusable prescription templates for common conditions to speed up consultations.

8.8.3 Drug Warnings

Automated warnings for known drug-drug interactions before a prescription is finalized.

8.9 Laboratory Module

8.9.1 Orders

Doctors create lab test orders directly from a consultation.

8.9.2 Results

Lab staff upload structured or scanned results against the order.

8.9.3 Abnormal Value Detection

Automated detection and highlighting of out-of-range lab values.

8.10 Billing Module

8.10.1 Invoices

Generate itemized invoices for consultations, tests, and procedures.

8.10.2 UPI Payments

Native UPI payment collection integrated into the billing flow.

8.10.3 Revenue Reports

Daily, weekly, and monthly revenue summaries for clinic owners.

8.11 Notification Module

8.11.1 SMS

Appointment reminders, OTPs, and key alerts via SMS.

8.11.2 Email

Transactional emails (e.g., invoices, reports, password resets).

8.11.3 Push Notifications

Real-time mobile push notifications for appointments, prescriptions, and reports.

8.12 Analytics Module

8.12.1 Clinic KPIs

Patient footfall, revenue trends, appointment no-show rates.

8.12.2 Doctor KPIs

Consultation volume, average consultation time, follow-up compliance.

8.12.3 Patient KPIs

Retention rate, repeat visit frequency, follow-up adherence.

8.13 Follow-Up Module

8.13.1 Chronic Disease Follow-Up

Structured follow-up schedules for chronic condition patients.

8.13.2 Reminder Engine

Automated reminders for upcoming or overdue follow-ups.

8.13.3 Recall Campaigns

Bulk recall campaigns for patient segments (e.g., overdue check-ups).

8.14 Patient Mobile App

8.14.1 Health Passport

Lifetime, chronological timeline of all medical interactions across linked clinics.

8.14.2 QR Identity

A scannable QR-based patient identity for quick check-in at any linked clinic.

8.14.3 Records Timeline

Browsable, filterable view of prescriptions, lab reports, and diagnoses.

8.14.4 AI Assistant

Conversational assistant answering questions strictly from the patient's own health records.

9. AI Requirements

9.1 Prescription OCR

Automatically extract structured medicine, dosage, and instruction data from handwritten or scanned prescriptions, converting them into digital records.

9.2 Medical Record Summaries

Generate concise, doctor-facing summaries of patient history, chronic conditions, current medications, and notable trends at the start of a consultation.

9.3 Drug Interaction Detection

Cross-check newly prescribed medicines against the patient's current medication list and flag potentially dangerous interactions before a prescription is finalized.

9.4 Lab Report Analysis

Automatically detect abnormal values within uploaded lab reports and generate a plain-language summary highlighting concerns for both doctor and patient.

9.5 AI Health Assistant

A patient-facing conversational assistant that answers questions exclusively from that patient's own verified health records (e.g., current medications, last visit's prescription, latest lab results). The assistant must not access or reference any other patient's data.

9.6 RAG Architecture

9.6.1 Embeddings

Patient health records (consultations, prescriptions, lab reports) are embedded and indexed for semantic retrieval.

9.6.2 Vector Database

A per-tenant/per-patient-scoped vector store ensures retrieval is strictly limited to the requesting patient's own data.

9.6.3 Retrieval Layer

A retrieval-augmented generation pipeline that fetches relevant record chunks before generating any AI assistant response, ensuring answers are grounded in actual patient data rather than model assumptions.

10. Non-Functional Requirements

10.1 Scalability

The system must scale horizontally to support a growing number of tenants, patients, and concurrent users without architectural rework, leveraging the modular monolith's clean module boundaries to later extract services if needed.

10.2 Availability

Target uptime of 99.5% or higher for core clinical workflows (appointments, consultations, prescriptions), as these directly impact patient care.

10.3 Performance

Key dashboard actions (patient search, appointment booking, prescription creation) should respond within 1–2 seconds under normal load.

10.4 Reliability

Critical data (prescriptions, lab results, billing) must be durably persisted with automated backups and disaster recovery procedures.

10.5 Maintainability

Modular monolith design with clear module boundaries (router, service, repository, models, schemas) to keep the codebase easy to understand, test, and extend.

11. Security Requirements

11.1 RBAC

Role-Based Access Control enforced for every action, scoped by role (Clinic Owner, Doctor, Receptionist, Lab Technician, Patient, System Administrator).

11.2 Audit Logs

Every action performed on patient data is logged with actor, timestamp, and action type for compliance and traceability.

11.3 Encryption

Sensitive data encrypted at rest and in transit; all traffic served over HTTPS.

11.4 File Security

Secure, access-controlled file uploads and storage for prescriptions, lab reports, and scanned documents.

11.5 Rate Limiting

API rate limiting to prevent abuse, brute-force attempts, and unintentional overload of shared resources.

12. Compliance Requirements

12.1 ABHA Readiness

Architecture and data models designed to support future linking with Ayushman Bharat Health Account (ABHA) IDs and the broader Ayushman Bharat Digital Mission (ABDM) ecosystem.

12.2 Consent Management

Explicit, patient-driven consent required before any cross-clinic record sharing or linking occurs; consent must be revocable.

12.3 Patient Privacy

Patient data is never shared with or accessible to unrelated clinics, third parties, or used for purposes beyond direct care without explicit consent.

12.4 Data Retention

Defined retention policies for clinical and billing data, aligned with applicable healthcare record-keeping regulations.

13. Architecture Overview

13.1 Modular Monolith

Aarogya360 follows a Modular Monolith architecture, chosen for faster development, simpler deployment, lower infrastructure complexity, and easier debugging compared to a premature microservices split.

13.2 Module Boundaries

The system is organized into the following modules: Authentication, Clinics, Doctors, Patients, Appointments, Consultations, Prescriptions, Billing, Notifications, AI Services, and ABHA Integration.

13.3 Service Layer Design

Each module follows a consistent internal structure:

- Router – API endpoint definitions
- Service – business logic
- Repository – data access layer
- Database Models – persistence schema
- Schemas – request/response validation

14. Data Model Overview

14.1 Core Entities

- **Clinic** – tenant entity representing a registered clinic
- **Doctor** – staff entity with specialty, availability, and consultation history
- **Patient** – individual with a lifelong health record, optionally linked across clinics
- **Appointment** – scheduled interaction between a patient and a doctor
- **Consultation** – record of symptoms, examination, diagnosis, and treatment plan
- **Prescription** – digital record of prescribed medicines linked to a consultation
- **Lab Report** – structured or uploaded test results linked to a lab order
- **Invoice** – billing record linked to a consultation or service rendered

15. API Requirements

15.1 Authentication APIs

Endpoints for registration, login, token refresh, password reset, and MFA verification.

15.2 Clinic APIs

Endpoints for clinic registration, staff management, and department configuration.

15.3 Doctor APIs

Endpoints for doctor profile management, availability configuration, and scheduling.

15.4 Patient APIs

Endpoints for patient registration, search, health passport retrieval, and family linking.

15.5 Appointment APIs

Endpoints for booking, rescheduling, queue management, and notification triggers.

15.6 AI APIs

Endpoints for prescription OCR submission, health summary generation, lab analysis, drug interaction checks, and the patient AI assistant query interface.

16. Reporting Requirements

16.1 Operational Reports

Appointment volumes, queue wait times, staff activity logs.

16.2 Clinical Reports

Consultation summaries, diagnosis trends, follow-up compliance rates.

16.3 Financial Reports

Revenue by service type, outstanding payments, UPI transaction summaries.

17. Product Roadmap

17.1 Phase 1: Core Foundation

Authentication, multi-tenancy, patient management, doctor management, and the appointment system.

17.2 Phase 2: Clinical Workflow

Consultations, prescriptions, lab orders, and billing.

17.3 Phase 3: Billing and Analytics

Advanced billing features, revenue reporting, and clinic/doctor/patient analytics dashboards.

17.4 Phase 4: Patient Mobile App

Health Passport, AI Assistant, notifications, and QR-based identity linking.

17.5 Phase 5: AI Services

OCR, medical record summaries, lab analysis, and drug interaction detection deployed at scale.

17.6 Phase 6: ABHA Integration

Health ID linking and integration with the National Health Ecosystem (ABDM).

18. Success Metrics

18.1 Product Metrics

Reduction in paper record usage, average consultation time, follow-up compliance rate.

18.2 Business Metrics

Number of paying clinic subscriptions, monthly recurring revenue (MRR), churn rate.

18.3 Adoption Metrics

Daily/monthly active clinics, daily/monthly active patient app users, AI feature usage rate.

19. Risks and Mitigation

19.1 Technical Risks

- **Risk:** AI features (OCR, drug interaction detection) produce inaccurate output.
Mitigation: Treat all AI outputs as advisory; require doctor confirmation before any AI-suggested action is finalized.
- **Risk:** Multi-tenant data isolation failure leading to cross-clinic data leakage.
Mitigation: Strict tenant-scoped queries, automated isolation testing, and regular security audits.

19.2 Regulatory Risks

- **Risk:** Delays or changes in ABHA/ABDM integration requirements.
Mitigation: Treat ABHA integration as a later phase; design data models to be ABHA-ready without blocking earlier phases.
- **Risk:** Evolving healthcare data protection regulations in India.
Mitigation: Maintain a compliance advisory relationship and review data handling policies periodically.

19.3 Operational Risks

- **Risk:** Low clinic adoption due to resistance to changing established workflows.
Mitigation: Prioritize ease of onboarding, staff training, and a clear demonstration of time savings.
- **Risk:** Dependence on a small team for a wide product scope.
Mitigation: Strict phase-based roadmap to avoid scope creep and ensure incremental, shippable progress.

20. Future Vision

20.1 Telemedicine

Enable remote video consultations integrated directly into the existing appointment and consultation workflow.

20.2 Pharmacy Integration

Connect digital prescriptions directly to partner pharmacies for fulfillment and delivery tracking.

20.3 Insurance Integration

Enable direct insurance claim submission and cashless treatment workflows linked to consultations and billing.

20.4 National Health Ecosystem

Deepen integration with ABHA and the broader Ayushman Bharat Digital Mission, positioning Aarogya360 as a fully interoperable node within India's national health data ecosystem.